

Artwork Document Information		Artwork Type [PLEASE TICK ✓]		Sign / Stamp	
Product : T-HYDRINATE SYRUP 15MG/5ML [FRONT] SAP No. : Date : 03 December 2024		☐ Unit Box ☐ Outer Box ☐ Label ☒ Leaflet ☐ Fix-A-Form ☐ Shipper Carton ☐ Aluminium Foil ☐ Sachet ☐ Others : _____			
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Hydrinate

Syrup 15mg/5ml



DESCRIPTION

Hydrinate Syrup 15mg/5ml
A light yellow, fruity flavoured syrup.
Each 5 ml contains Dimenhydrinate 15 mg.
Preservatives: Methylparaben 0.21% w/v
Propylparaben 0.031% w/v

INDICATIONS

For prevention and treatment of motion sickness. Also used as antiemetic agent in irradiation sickness, postoperative vomiting, drug induced nausea and vomiting, and for symptomatic treatment of nausea and vertigo due to Meniere's disease and other labyrinthine disturbances.

PHARMACODYNAMICS

The mechanism by which dimenhydrinate exerts its antiemetic, antimo-tion sickness and antivertigo effects is not precisely known but may be related to their central anticholinergic actions. They diminish vestibular stimulation and depress labyrinthine function. An action on the medullary chemoreceptive trigger zone may also be involved in the antiemetic effect. Dimenhydrinate also has anticholinergic and local anaesthetic effects. Its use in the treatment of allergy is by competing with histamine H1-receptor sites on effector cells. It does not inactivate histamine or prevent its synthesis or release. It thereby prevents, but does not reverse, responses mediated by histamine alone. Antihistamines antagonise, in varying degrees, most of the pharmacological effects of histamine, including urticaria, pruritus and vasodilation.

PHARMACOKINETICS

Dimenhydrinate is well absorbed after oral administration. It is metabo-lised in the liver by the cytochrome P-450 enzyme system. The onset of action of dimenhydrinate is between 15-60 minutes. A duration of action of 3-6 hours has been reported. Dimenhydrinate is mainly excreted as its metabolites in urine within 24 hours.

DOSAGE

Adult

50-100 mg every 4 hours as needed.
For motion sickness, take medicine at least 30 minutes, and preferably 1-2 hours before travelling.
Usual adult prescribing limit: Up to 400 mg daily.

Pediatric

Children 6-12 years: 25-50 mg every 6-8 hours as needed (maximum of 150 mg daily).

Geriatric

Usual adult dose.
Geriatric patients may be more sensitive to the effects of the usual adult dose.

SIDE EFFECTS

Drowsiness, sedation and thickening of mucous occurs frequently with dimenhydrinate. Blurred vision, confusion, difficult or painful urination, dizziness, dry mouth, nose, or throat, tachycardia, increased sweating, loss of appetite, paradoxical reactions, photosensitivity, ringing or buzzing in ears, skin rash, stomach upset and constipation have also been reported. These effects are more likely to occur in children and elderly patients. Blood dyscrasias including agranulocytosis, leucopenia, haemolytic anaemia, and thrombocytopenia though rare, have been reported. Other rare side effects include hypotension, headache and paraesthesia.

CONTRAINDICATIONS

Dimenhydrinate should not be given to premature infants or neonates; this group of patients has an increased susceptibility to antimuscarinic effects.

PRECAUTIONS

The anticholinergic properties of dimenhydrinate may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients which may seriously affect ability to drive or operate machinery. Anticholinergic effects may also precipitate or aggravate urinary retention in patients with bladder neck obstruction, symptomatic prostatic hypertrophy and predisposition to urinary retention. Increased intraocular pressure may precipitate an attack of angle-closure glaucoma. Dimenhydrinate should be used with care in patients with pyloroduodenal obstruction, severe cardiovascular disorders and epilepsy. Patients sensitive to one antihistamine may be sensitive to dimenhydrinate as well. Caution is also recommended when dimenhydrinate is used, since their antiemetic action may impede diagnosis of such conditions as appendicitis and obscure signs of toxicity from overdosage of other drugs. Dimenhydrinate may inhibit the cutaneous histamine response, thus producing false-negative results; it is recommended that dimenhydrinate be discontinued at least 72 hours before testing begins.

DRUG INTERACTIONS

Concurrent use with alcohol and CNS depressants such as barbiturates, hypnotics, opioid analgesics, monoamine oxidase (MAO) inhibitors, anxiolytic sedatives and neuroleptics may potentiate the CNS depressant effects. Anticholinergic effects may be potentiated when these medications such as atropine, tricyclic antidepressants and MAO inhibitors are used concurrently with antihistamines. Concurrent use with ototoxic medications such as aminoglycoside antibiotics may mask the symptoms of ototoxicity such as tinnitus, dizziness and vertigo. Additive photosensitising effects may occur with use of these medications with antihistamines. Prior administration of dimenhydrinate may decrease the emetic response to apomorphine in the treatment of poisoning.

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OVERDOSAGE

Symptoms

Clumsiness or unsteadiness, severe drowsiness, severe dryness of mouth, nose or throat, flushing or redness of face, shortness of breath or troubled breathing are signs and symptoms of anticholinergic effects. Cardiac arrhythmia, CNS stimulation and hypotension are all signs and symptoms of overdosage. Anticholinergic and CNS stimulant effects are more likely to occur in children with overdose while hypotension may also occur in the elderly at usual doses. Overdose may be fatal especially in infants and children.

TREATMENT

Since there is no specific antidote for overdose with dimenhydrinate, treatment is symptomatic and supportive with possible utilisation of the following:

- Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children.
- Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.
- Saline cathartics (milk of magnesia) and activated charcoal are sometimes used, especially if sustained-release preparations were involved.
- Vasopressors such as norepinephrine or phenylephrine to treat hypotension; however, epinephrine should not be used since it can paradoxically lower the blood pressure further.
- Oxygen, external cooling for hyperpyrexia and intravenous fluid.
- Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Up-to-date information on treatment of overdose can be obtained from the National Poison Center, University Sains Malaysia (Tel: 800-8099)

TOXICOLOGY

Cross sensitivity and / or related problems

Patients sensitive to one of the antihistamines may be sensitive to others.

Carcinogenicity/Tumorigenicity/Mutagenicity

Long term animal studies to evaluate carcinogenic, tumorigenic, or mutagenic potential of most antihistamines have not been performed.

Pregnancy/Reproduction

Studies with dimenhydrinate in humans have not been done. Studies in animals have not shown that dimenhydrinate causes adverse effects on the fetus. (FDA Pregnancy Category B)

Breast-feeding

First generation antihistamines may inhibit lactation because of their anticholinergic actions. Small amounts of antihistamines are distributed into breast milk; use is not recommended in nursing mothers because of the risk of adverse effects in infants, such as unusual excitement or irritability.

Pediatrics

Use is not recommended in newborn or premature infants because this age group has increased susceptibility to anticholinergic side effects, such as

central nervous system (CNS) excitation, an increased tendency towards convulsion. A paradoxical reaction characterized by hyperexcitability may occur in children taking antihistamines.

Geriatrics

Dizziness, sedation, confusion, and hypotension may be more likely to occur in geriatric patient taking antihistamines. A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines. Geriatric patients are especially susceptible to the anticholinergic side effects such as dryness of mouth and urinary retention (especially in males) of the antihistamines. If these side effects occur and continue or are severe, medication should probably be discontinued.

Dental

Prolonged use of antihistamines may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

TREATMENT

90ml

STORAGE CONDITIONS AND USER INSTRUCTIONS

Keep container tightly closed (syrup only).
Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhi daripada kanak-kanak.
Shelf life: please refer to outer package.
Hydrinate may be taken with food, water or milk.
Do not take more than the amount recommended.
May cause drowsiness. If affected, do not drive or operate machinery.
Avoid alcoholic beverages.

Route of administration: Oral

Product Registration Holder:

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